BASELINE → TREATMENT → FOLLOW-UP

# When checks happen

Milestones show order, not equal time intervals or booked appointments. The treating team requests tests, reviews results and confirms treatment. FBC, renal and liver profile before each Day 1, Day 8 and Day 15 treatment, not only at the start of the 28-day block. Assess neuropathy and function before doses. Day 22 is a planned week without paclitaxel in 00621. The separate 00226 weekly alternative must not silently add a fourth dose to this card.

| Check | Before starting | Early treatment | Ongoing treatment | Dose change / pause | Next decision |
|---|---|---|---|---|---|
| **Blood / kidney / liver** | Baseline FBC, kidney and liver tests | FBC, kidney and liver tests before Days 1, 8 and 15 | FBC, kidney and liver tests before Days 1, 8 and 15 | Recheck recovery and dose before restarting | Follow unresolved abnormalities |
| **Symptoms / medicines** | Review symptoms, medicines and previous treatment | Review effects before each dose | Continue symptom and medicine reviews | Report problems; do not change or restart doses yourself | Agree recovery and next treatment plan |
| **Heart / ECG / echo** | Cardiac history, symptoms and infusion observations guide assessment. Do not add the trastuzumab three-month echo schedule to paclitaxel monotherapy. | Review symptoms and indicated tests | Continue the risk- and treatment-specific plan | Urgent assessment for chest pain, fainting or breathlessness | Agree follow-up from exposure and results |
| **Response / course** | Confirm cancer-specific treatment aim | Three weekly doses and a planned week without treatment | Review benefit, symptoms and the agreed scan plan | Reassess if treatment is held or symptoms change | Agree continuation, change or completion |
| **Bone health / other care** | Review previous/ongoing treatments and fracture risk | No automatic chemotherapy-cycle DXA | Keep any separate endocrine/ fracture-risk plan | Review if clinical risks change | Continue separately indicated follow-up |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00621 v3 · official regimen · HPRA conventional paclitaxel product information; 09 January 2026

THE WHOLE TREATMENT JOURNEY

# Paclitaxel · Days 1, 8 and 15 of 28: treatment and checks

**Source-defined advanced gastric, breast, ovarian, small-cell lung or bladder indications**

Paclitaxel 80 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Day 22 is a planned week off, not a fourth routine dose. Continue according to benefit and tolerance. The NCCP indications are off-label in Ireland and require the local consent / approval process.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Early treatment | Ongoing treatment | Dose change / pause | Next decision |
|---|---|---|---|---|---|
| **Treatment plan** NCCP 00621 | Confirm indication, goal and current prescription | Three weekly doses and a planned week without treatment | Continue according to response, tolerance and prescribed course | Team review before restart or dose change | Agree completion or next treatment |
| **Blood / kidney / liver** NCCP 00621 · SMPC | Baseline FBC, kidney and liver tests | FBC, kidney and liver tests before Days 1, 8 and 15 | FBC, kidney and liver tests before Days 1, 8 and 15 | Recheck recovery and dose before restarting | Follow unresolved abnormalities |
| **Symptoms / medicines** NCCP 00621 · SMPC | Review symptoms, medicines and previous treatment | Review effects before each dose | Continue symptom and medicine reviews | Report problems; do not change or restart doses yourself | Agree recovery and next treatment plan |
| **Heart / ECG / echo** NCCP 00621 · SMPC | Cardiac history, symptoms and infusion observations guide assessment. Do not add the trastuzumab three-month echo schedule to paclitaxel monotherapy. | Review symptoms and indicated tests | Continue the risk- and treatment-specific plan | Urgent assessment for chest pain, fainting or breathlessness | Agree follow-up from exposure and results |
| **Response / course** NCCP 00621 · SMPC | Confirm cancer-specific treatment aim | Three weekly doses and a planned week without treatment | Review benefit, symptoms and the agreed scan plan | Reassess if treatment is held or symptoms change | Agree continuation, change or completion |
| **Bone health / other care** NCCP 00621 · SMPC | Review previous/ongoing treatments and fracture risk | No automatic chemotherapy-cycle DXA | Keep any separate endocrine/ fracture-risk plan | Review if clinical risks change | Continue separately indicated follow-up |

# What each check is for

### 1 Treatment and safety review

Before each planned treatment or four-week review for olaparib

FBC, renal and liver profile before each Day 1, Day 8 and Day 15 treatment, not only at the start of the 28-day block. Assess neuropathy and function before doses.

**Ask / act:** The treating team confirms results and the plan; this map does not authorise a dose.

Sources: NCCP 00621 · SMPC

### 2 Benefit and continuation

At agreed clinical review points

Paclitaxel may slow or shrink metastatic breast cancer and help symptoms. This prescribed pattern includes a week without paclitaxel. There is no verified numerical survival estimate for this exact schedule in this review; results from a different weekly pattern do not predict your outcome.

**Ask / act:** Discuss benefit, toxicity and priorities before continuing or changing treatment.

Sources: NCCP 00621 · RETROSPECTIVE-TRIAL

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.21